

# Intro to AI for Medicine

Lec 4: Training

# Last Time

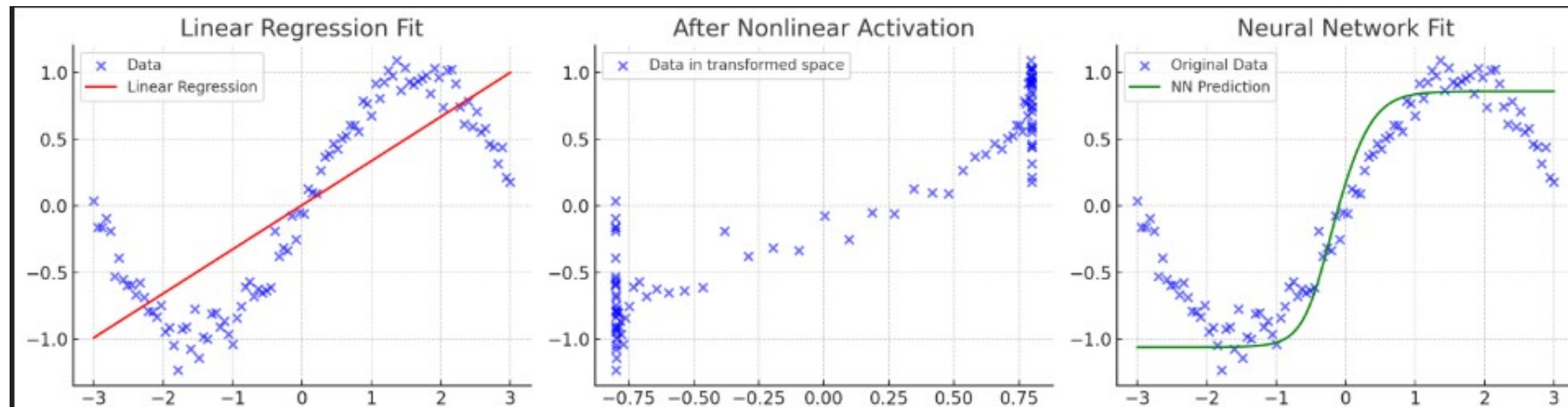
- Learned about loss functions, finishing neural net structure
- Defined regression, classification (including multiclass)
- Associated final activation with loss function and problem type
- Discussed some metrics for measuring how good our networks are

# Agenda

- Overfitting v underfitting
- Internal/external validity
- Hyperparameters
- Revisit GD
- Ensembling
- Case study – afib ablations w/wo AI tool RCT

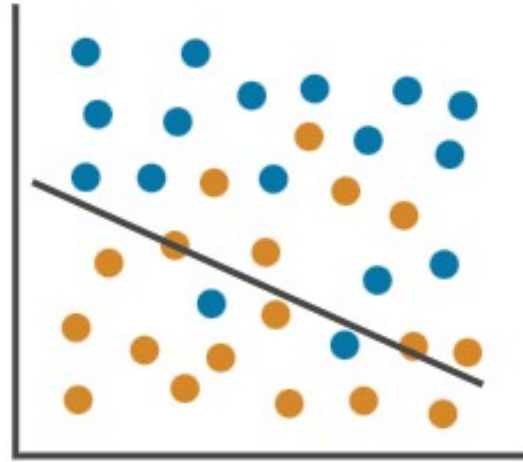
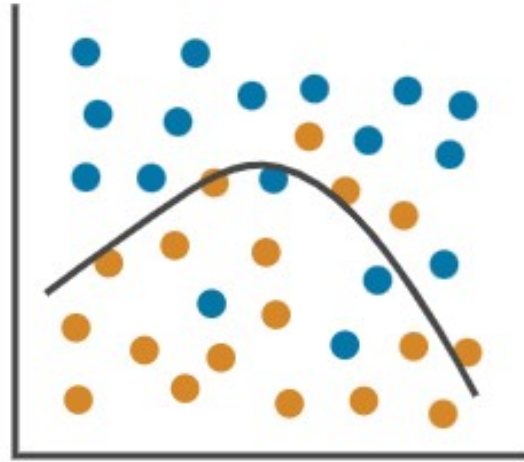
# So Far...

- We've seen plots like below, and assumed a good fit
- How can we leverage metrics to tell us if we're really fitting the trend?
- How do we know if our model will work on other populations?



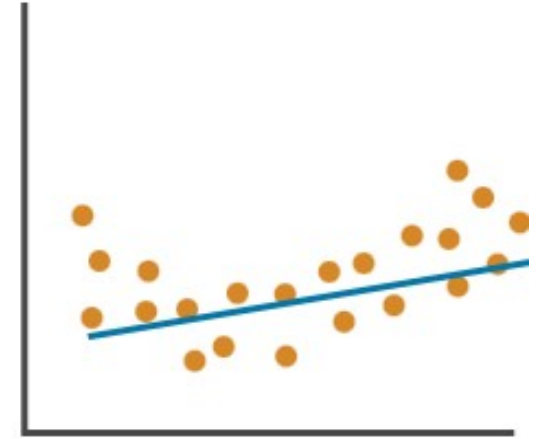
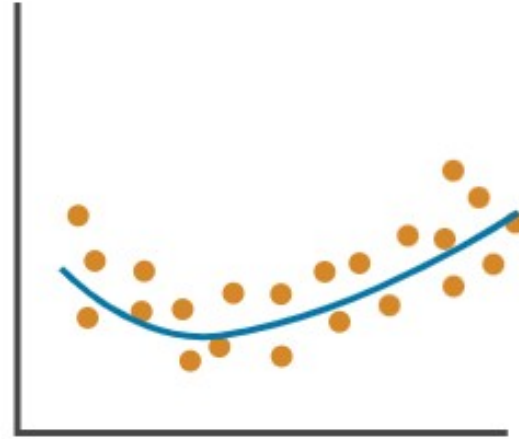
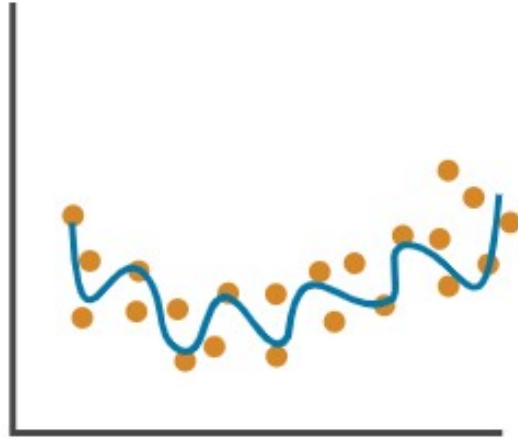
# Fitting Data

Classification



# Fitting Data

Regression



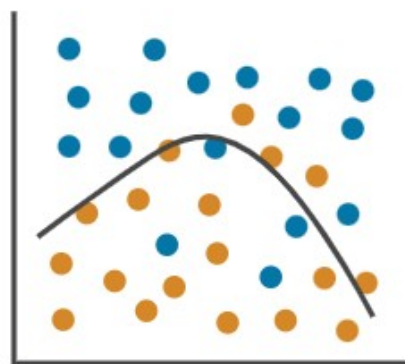
# Fitting Data

Classification

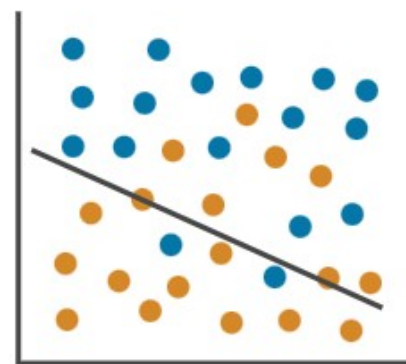
Overfitting



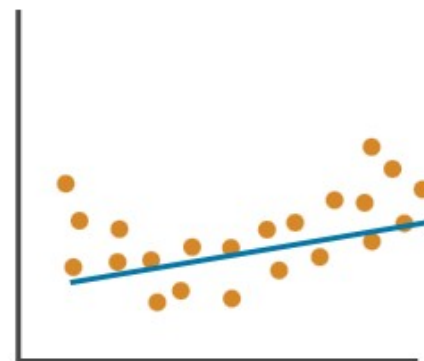
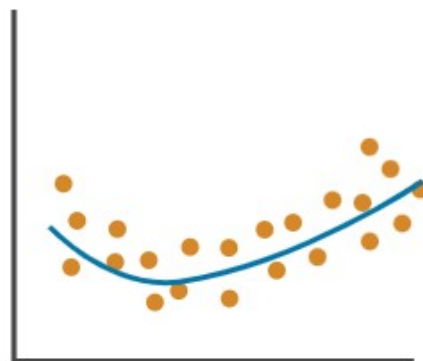
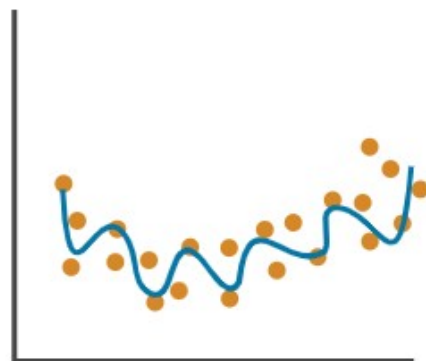
Right Fit



Underfitting

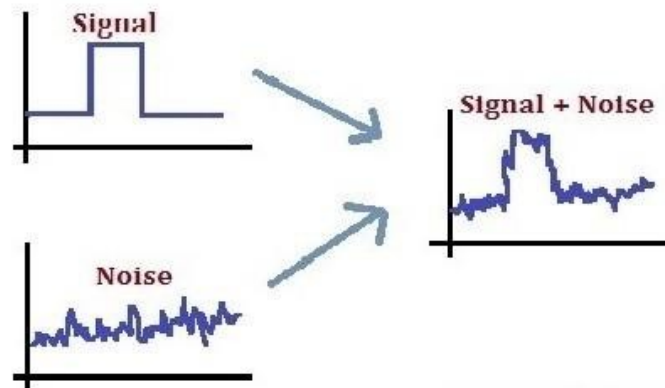


Regression



# Overfitting

- Overfitting: model is so closely aligned to the training data that it does not know how to respond to new data.
- Overfitting can happen because:
  - model is too complex; it memorizes very subtle patterns in the training data that don't generalize well.
  - The training data size is too small for the model complexity and/or contains large amounts of irrelevant information.





# Underfitting

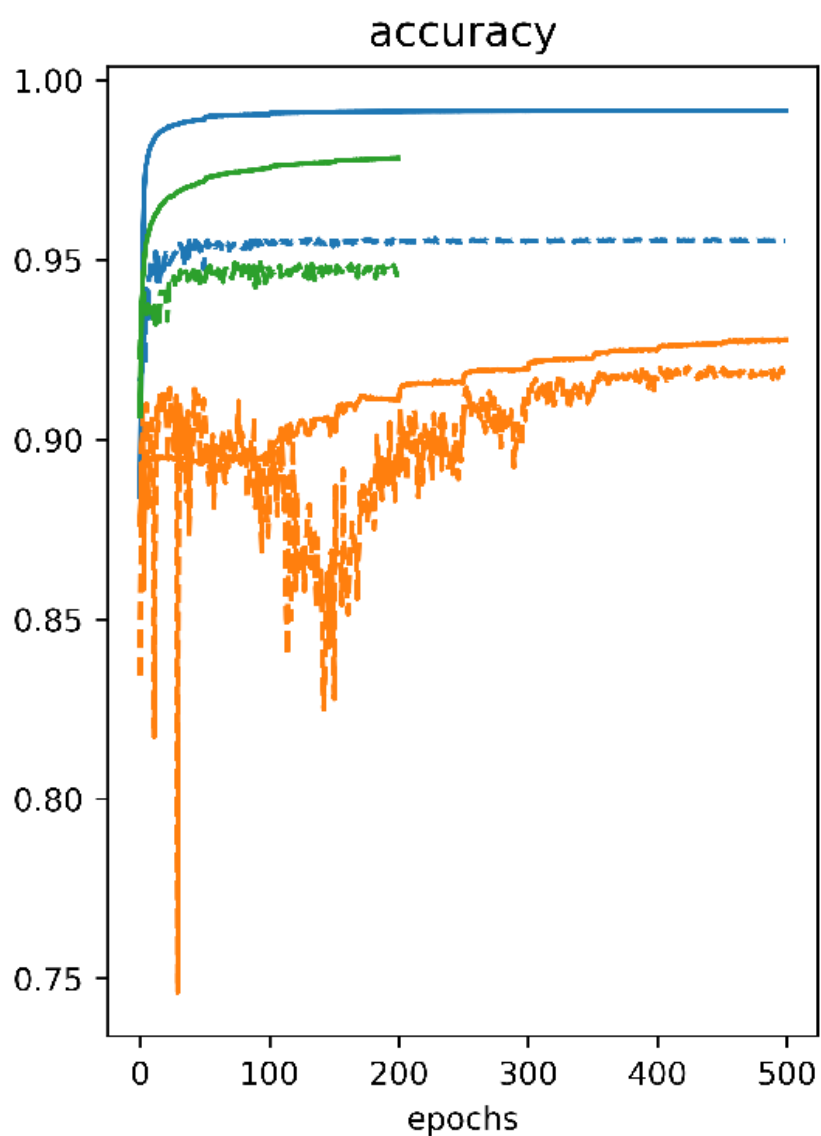
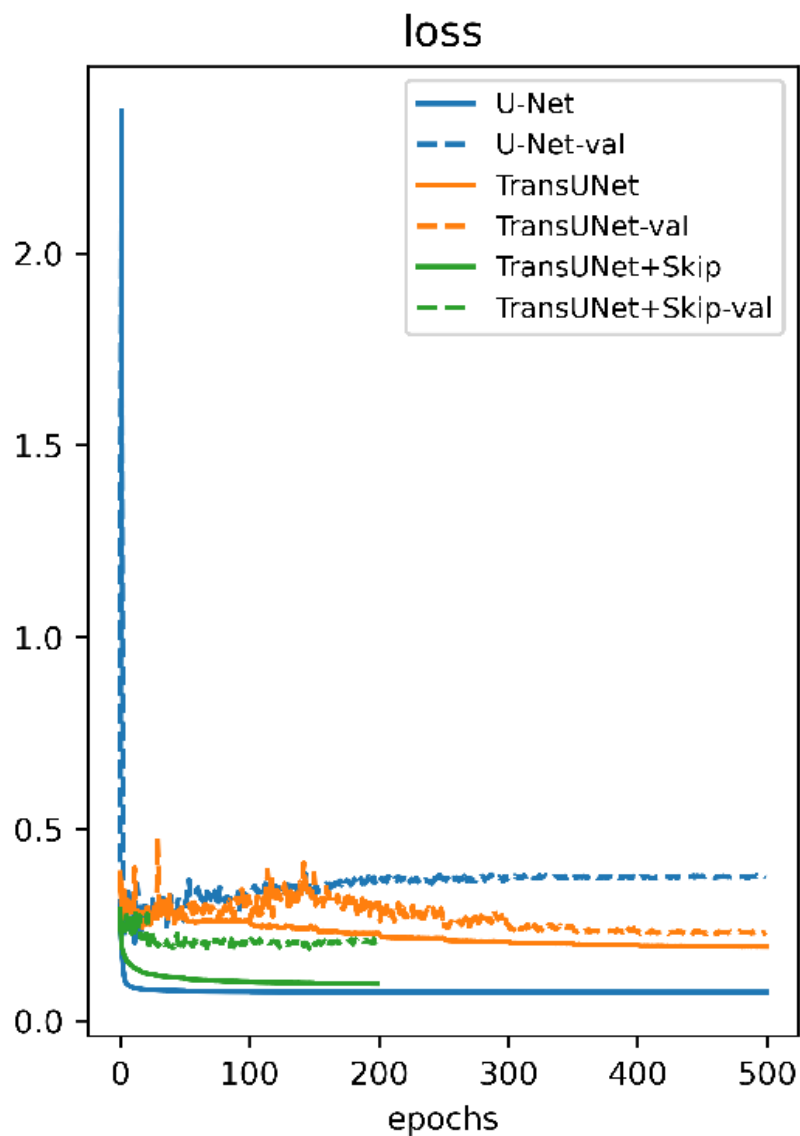
- The model couldn't see the trend
- Model is too simple to capture the underlying patterns and complexities of the data

# Did We Overfit?

(loss)

| Error    | Overfitting                                                                         | Right Fit | Underfitting                                                                         |
|----------|-------------------------------------------------------------------------------------|-----------|--------------------------------------------------------------------------------------|
| Training |  | Low       |  |
| Test     |  | Low       |  |

# Training vs Test Loss and Accuracy



1 Epoch:  
1 pass over all  
training samples

# Training in Real Time

```
Epoch 06 | Train loss 0.6990, Train acc 0.333, Val acc 0.800, Val AUC 0.250, F1 0.000, Precision 0.000, Recall 0.000

Train:  0%|          | 0/9 [00:00<?, ?it/s]
Train: 11%|█         | 1/9 [00:00<00:06, 1.15it/s]
Train: 22%|██        | 2/9 [00:01<00:05, 1.32it/s]
Train: 33%|███       | 3/9 [00:02<00:04, 1.39it/s]
Train: 44%|████      | 4/9 [00:02<00:03, 1.42it/s]
Train: 56%|█████     | 5/9 [00:03<00:02, 1.44it/s]
Train: 67%|██████    | 6/9 [00:04<00:02, 1.45it/s]
Train: 78%|███████   | 7/9 [00:04<00:01, 1.46it/s]
Train: 89%|████████  | 8/9 [00:05<00:00, 1.46it/s]
Train: 100%|█████████| 9/9 [00:06<00:00, 1.47it/s]

Val:  0%|          | 0/5 [00:00<?, ?it/s]
Val: 20%|█         | 1/5 [00:00<00:00, 6.77it/s]
Val: 40%|██        | 2/5 [00:00<00:00, 6.97it/s]
Val: 60%|███       | 3/5 [00:00<00:00, 7.08it/s]
Val: 80%|████      | 4/5 [00:00<00:00, 7.09it/s]
Val: 100%|██████   | 5/5 [00:00<00:00, 7.05it/s]

Epoch 07 | Train loss 0.6976, Train acc 0.333, Val acc 0.800, Val AUC 0.250, F1 0.000, Precision 0.000, Recall 0.000

Train:  0%|          | 0/9 [00:00<?, ?it/s]
Train: 11%|█         | 1/9 [00:00<00:06, 1.16it/s]
Train: 22%|██        | 2/9 [00:01<00:05, 1.33it/s]
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Train: 100%|█████████| 9/9 [00:06<00:00, 1.47it/s]
```

# Intuition For Fit

- Small models vs large models -> which would be more likely to overfit or underfit?

# Checking External Validity

- Divide data into parts
- Train data: Data we actually use to check loss and update weights
- Test data: Data we then use to compute accuracy to see if our model works on new data

# Internal v External Validity

- Internal validity: Is the model a good reflection of the data?
- External validity: Can the model be used on other data?

# Train/Test Splitting

- What if we get a non-representative train/test split?
- Cross validation
  - Divide data into a certain number of groups
    - K of them
    - Each group is called a “fold”
  - Take turns on which group is test data, the rest are training data
  - Average accuracy across runs

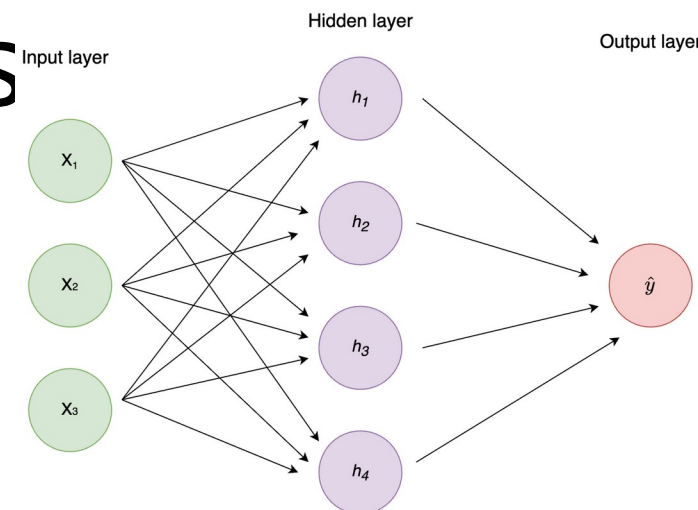


# Hyperparameters

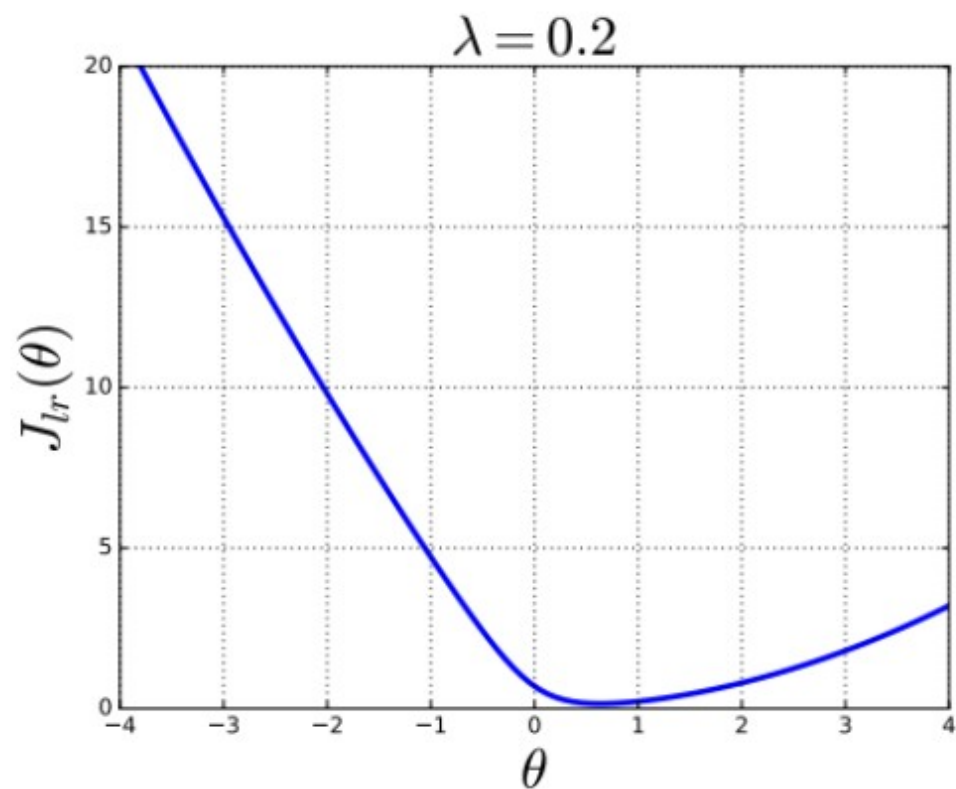
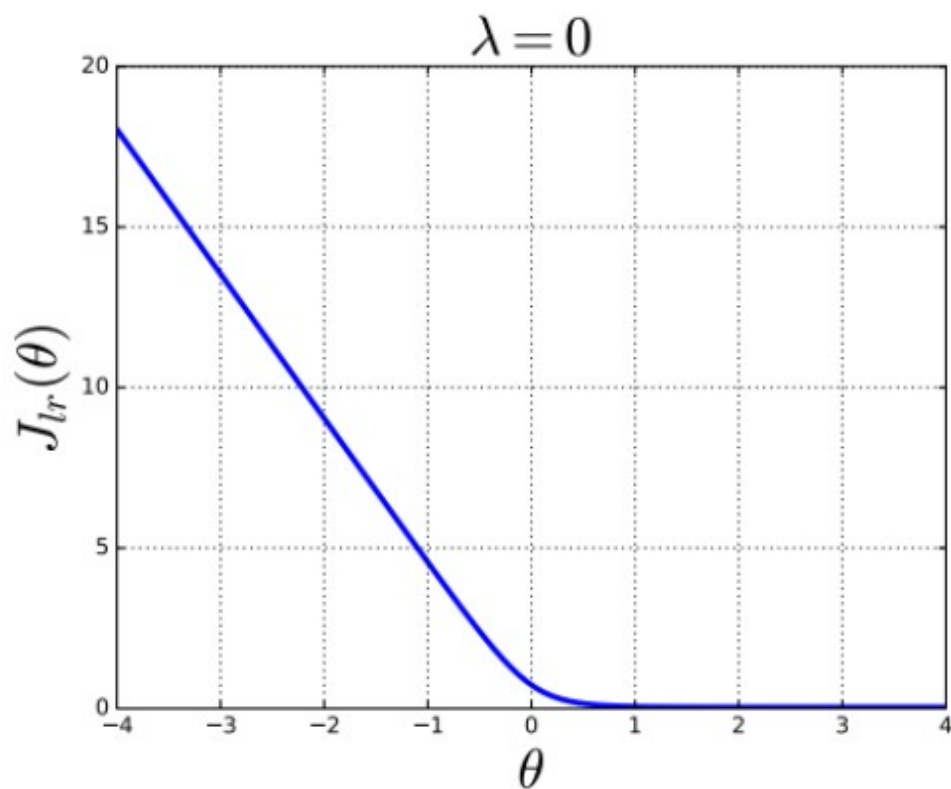
- Values that are **chosen**, not learned
- Contrast with parameters, which are the weights improved via gradient descent

# Hyperparameters - Examples

- Mandatory
  - Initial weights
  - Learning rate
  - Stopping condition
- Network architecture (covered in 2 lectures)
  - Depth of the network (how many layers)
  - Thickness of the network (how many neurons in parallel per layer)
- Data related (next lecture)
  - Augmentations
- Loss function bias
  - Batch norming
  - Dropout
  - Regularization
  - Many, many more



# Regularization



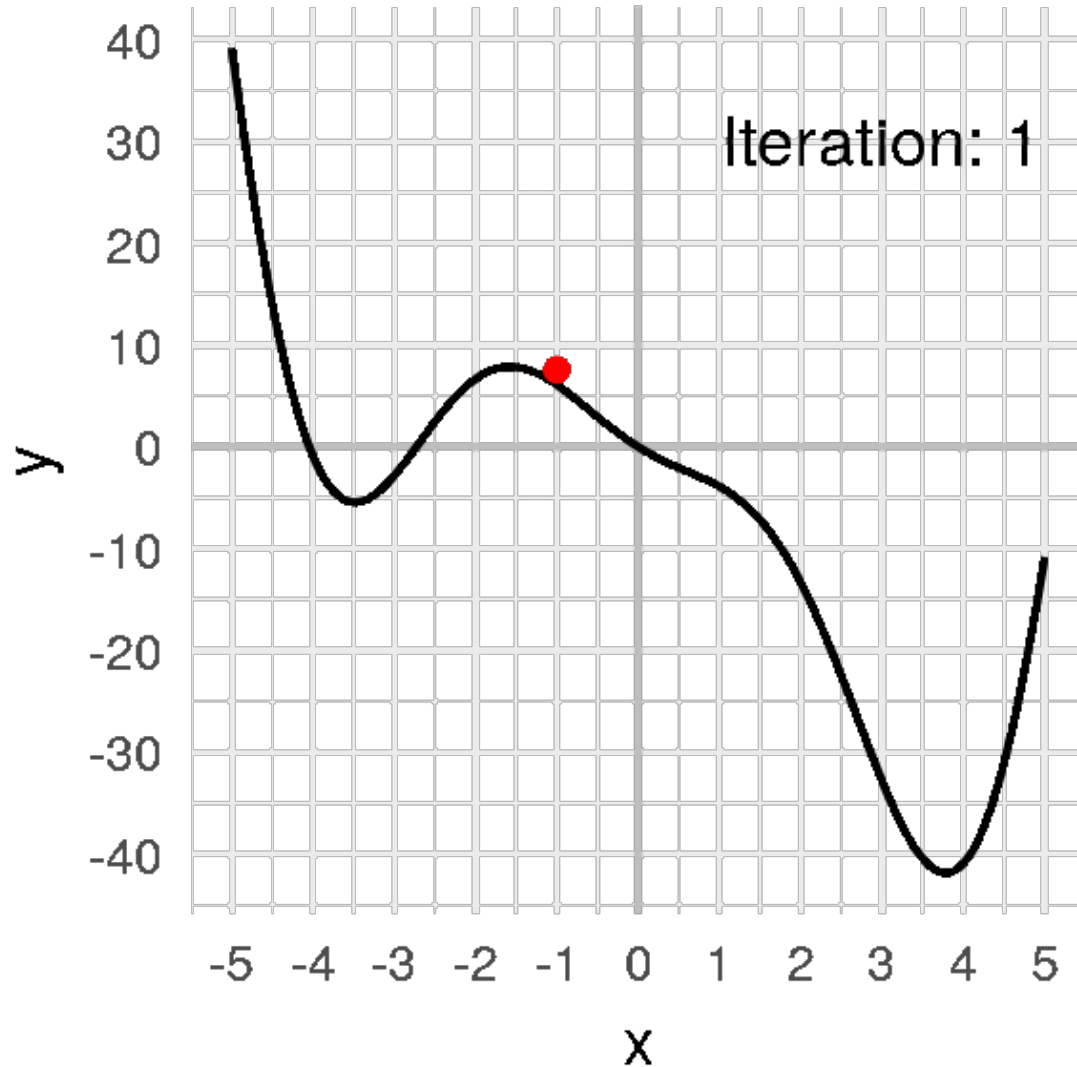
$$J_{lr}(\theta, \theta_0; \mathcal{D}) = \left( \frac{1}{n} \sum_{i=1}^n \mathcal{L}_{nll}(\sigma(\theta^T \mathbf{x}^{(i)} + \theta_0), \mathbf{y}^{(i)}) \right) + \lambda \|\theta\|^2$$

# Ridge Regression

- Regression with a regularization term
- Lasso the same but without the square (absolute)
  - Tends to remove many parameters from the model

$$J_{\text{ridge}}(\theta, \theta_0) = \frac{1}{n} \sum_{i=1}^n \left( \theta^T \mathbf{x}^{(i)} + \theta_0 - y^{(i)} \right)^2 + \lambda \|\theta\|^2$$

# Gradient Descent - Revisit

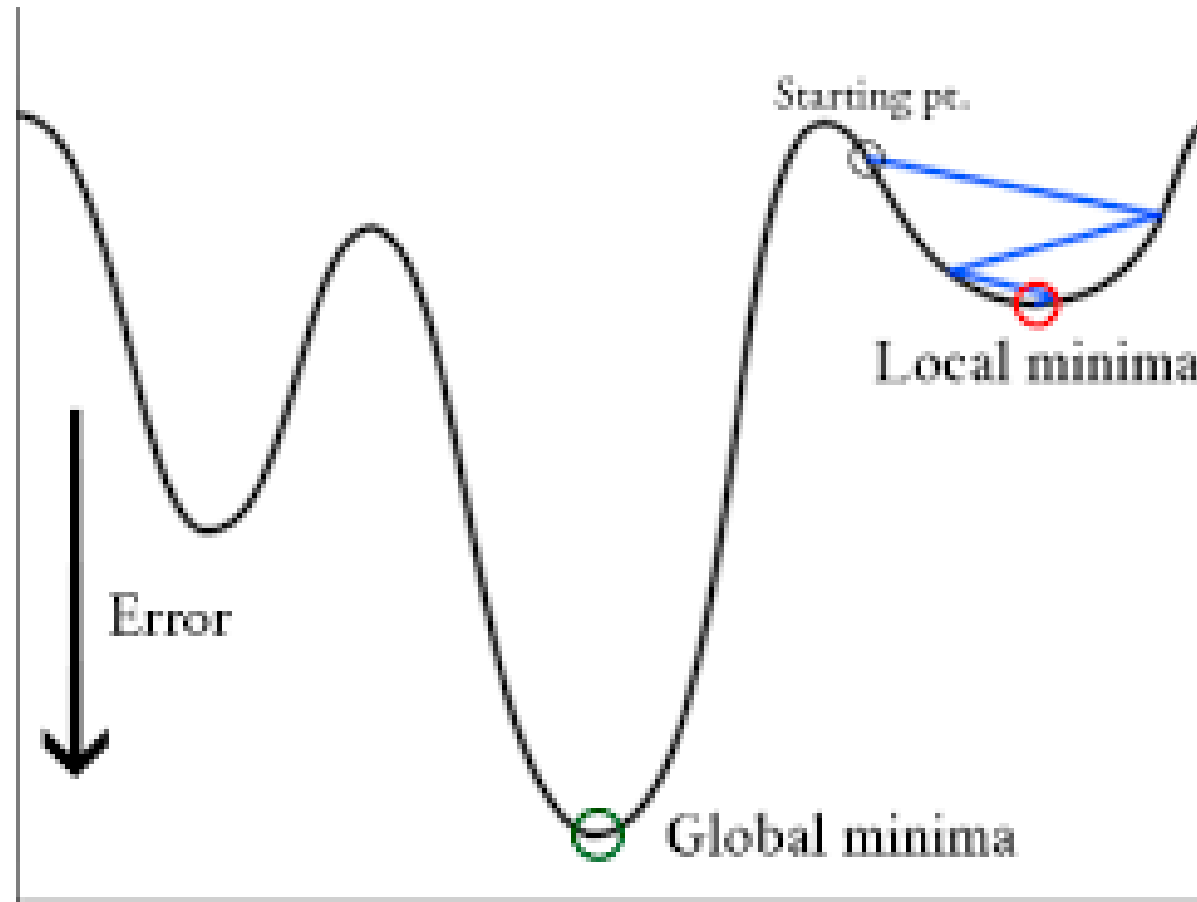


Algorithm:

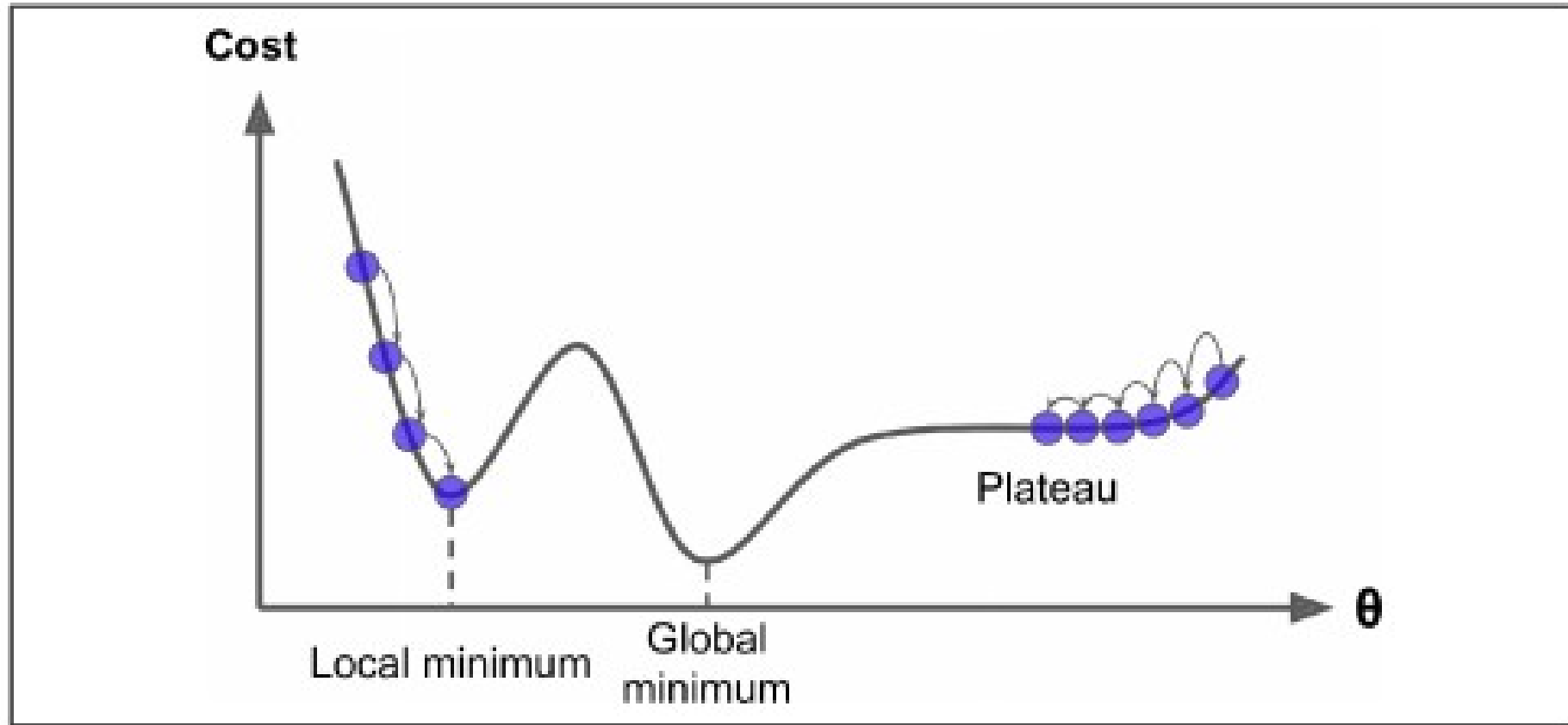
1. Find gradient at start point
2. Move opposite that direction by an amount = gradient magnitude times a constant (learning rate)
3. Repeat until the value between steps changes less than a set amount (or a fixed number of times)

$$\mathbf{a}_{n+1} = \mathbf{a}_n - \eta \nabla f(\mathbf{a}_n)$$

# Gradient Descent - Revisit



# Gradient Descent - Revisit



# Gradient Descent - Revisit

- Takeaway: All AI algorithms are, by definition, **approximations**
- Lots of work is happening under the hood to help us find the global min/max
- Often worth retraining networks with different parameters if result isn't good
- Choice of hyperparameters can greatly affect your loss



# Stochastic GD

- Avoiding pitfalls in GD via introducing random element
- Instead of descending down gradient of all points together, choose 1 point and make weights better for just that point
- On average, processes like the normal way would

# Vanishing Gradient Problem

- Over many layers, gradient can either blow up (if values large) or vanish (if values close to 0)
- Many things under the hood happening to prevent this
  - Batch norm
  - Choice of activation (ReLU safer than Sigmoid, Tanh)

# Ensembling

- Methods for training multiple models on same data, then averaging or combining in smart way to get better performance
- Bootstrapping: Training many models and averaging their results
  - Build datasets by sampling w replacement, then train
  - Often used in cases with little data
- Boosting:
  - Train model, then train new model whose goal is to take output of first one and reduce its loss when applied to data
- Random Forest
  - To be discussed later

# Case Study

[nature](#) > [nature medicine](#) > [articles](#) > article

Article | [Open access](#) | Published: 14 February 2025

## **Artificial intelligence for individualized treatment of persistent atrial fibrillation: a randomized controlled trial**

[Isabel Deisenhofer](#) , [Jean-Paul Albenque](#), [Sonia Busch](#), [Edouard Gitenay](#), [Stavros E. Mountantonakis](#), [Antoine Roux](#), [Jerome Horvilleur](#), [Babe Bakouboula](#), [Saumil Oza](#), [Selim Abbey](#), [Guillaume Theodore](#), [Antoine Lepillier](#), [Yves Guyomar](#), [Francis Bessiere](#), [Jaap Jan Smit](#), [Theophile Mohr Durdez](#), [Paola Milpied](#), [Anthony Appetiti](#), [Daniel Guerrero](#), [Tom De Potter](#), [Christian De Chillou](#), [Seth Goldberg](#), [Atul Verma](#), [John D. Hummel](#) & [TAILORED-AF Investigators](#)

[Nature Medicine](#) **31**, 1286–1293 (2025) | [Cite this article](#)

|                                                                                                                            |                                                                                                                                                                                                   |
|----------------------------------------------------------------------------------------------------------------------------|---------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| General Problem Statement                                                                                                  |                                                                                                                                                                                                   |
| Measure afib recurrence in 2 groups, one informed by AI, one not                                                           |                                                                                                                                                                                                   |
| Technical Problem Statement                                                                                                |                                                                                                                                                                                                   |
| RCT comparing humans to classification model, which predicts regions which should be ablated                               |                                                                                                                                                                                                   |
| Data                                                                                                                       | Loss                                                                                                                                                                                              |
| Proprietary dataset of 275k (train) and 113k (test) mappings<br>Annotations by EP doctors in France/USA                    | Not listed – assume creative combo of cross entropy with DICE or other 3d loss d/t problem construction (classification+CNN)<br>Study aim: Compare inter observer variability to network/observer |
| Network                                                                                                                    | Training                                                                                                                                                                                          |
| Multimodal network integrating <ol style="list-style-type: none"> <li>Classifier</li> <li>Convolutional network</li> </ol> | Single train/test split<br>Extreme gradient boosting (ensembling)                                                                                                                                 |

# RCT Design

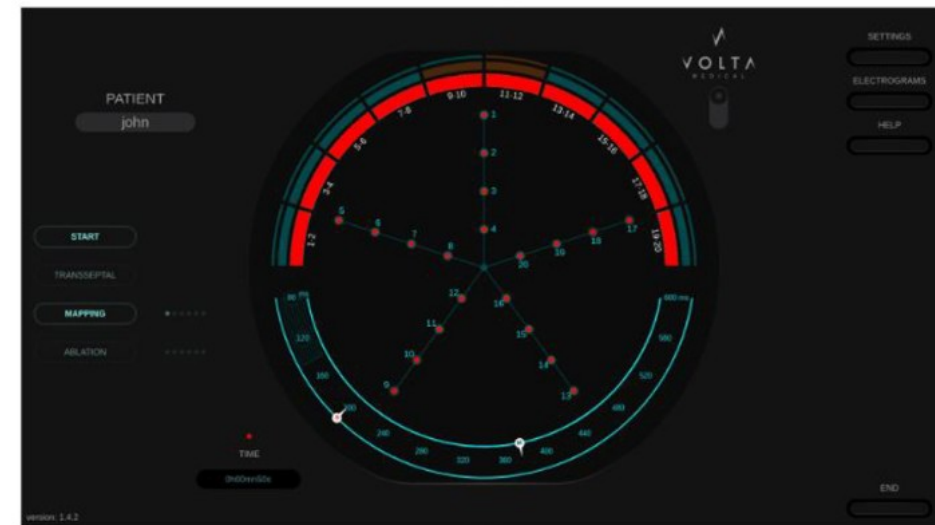
- 370 patients underwent catheter ablation
- Divided randomly into 2 groups: 1 with AI assistance during procedure, 1 without
  - Assistance is highlighted locations which might represent “dispersion” or sites from which electrical waves originate
  - Essentially sites of dispersion become ablation targets

# Algorithm Results

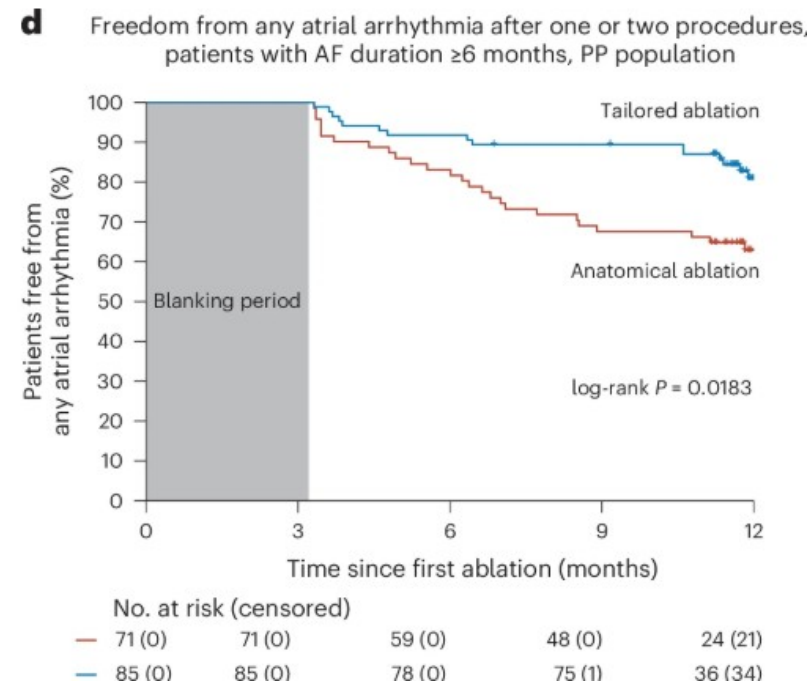
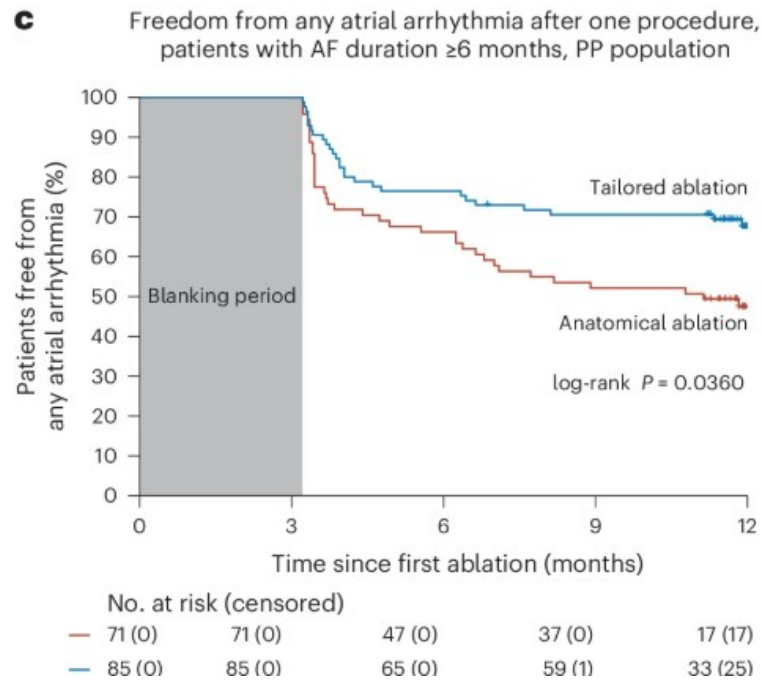
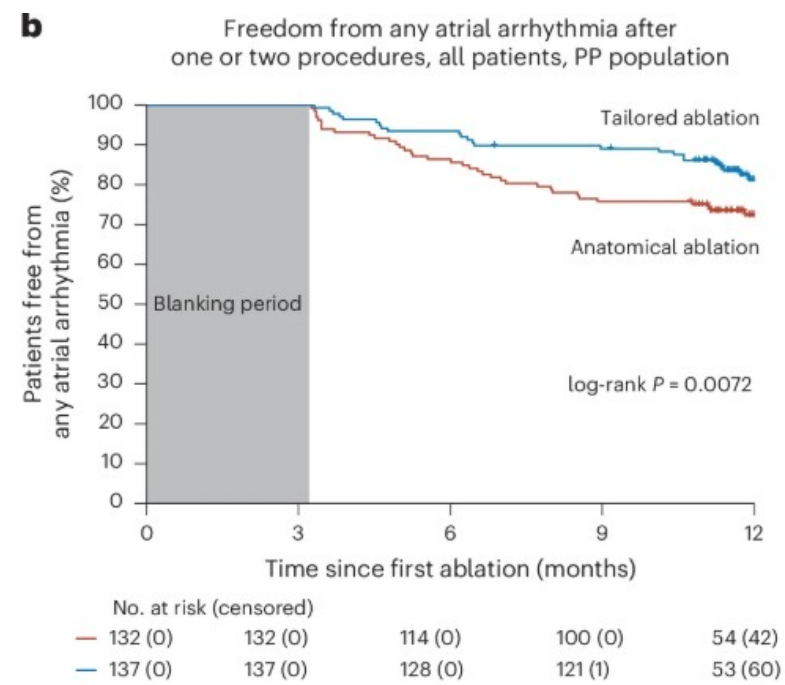
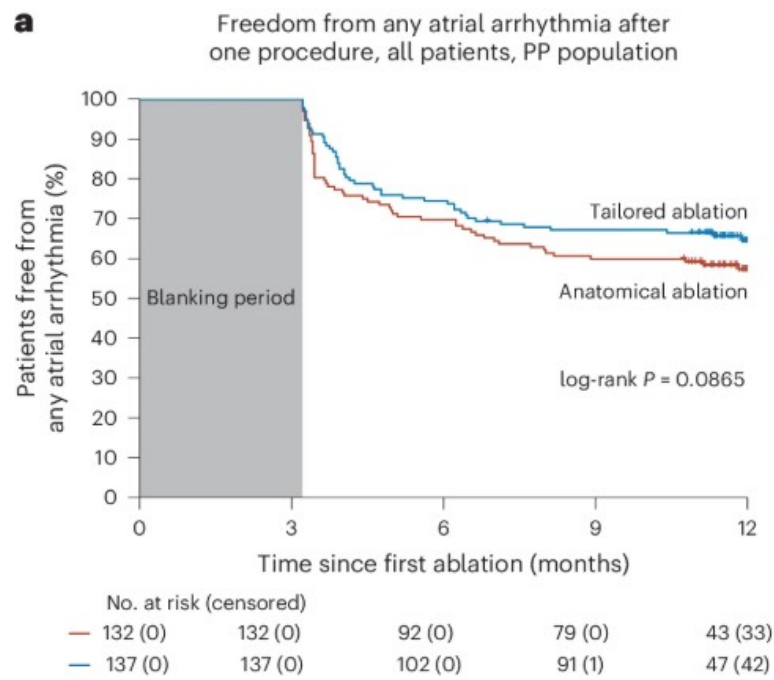
- AUROC = .93 -> better when experts all agreed on location of dispersion or lack thereof



## Detection of dispersed EGMs



**Catheter Ablation**





# Takeaways

- Authors have clear conflict of interest
- With that said, results were significant in 2 ablation, and sustained Afib groups
  - Of course, be skeptical of subgrouping
- Noted qualitative increase in complication rate in AI assisted group
- As algorithms improve could be a useful tool

# Recipe for Training

1. Problem formulation
2. Mapping between inputs and outputs
3. Choose loss function (often informed by problem formulation)
4. Train/test split
5. Train according to loss
6. Repeat training and average across many runs
7. Adjust hyperparameters as needed and repeat